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Source Motion PT

Complete User Guide

Covers: Getting Started · Scheduling · Patient Management ·
Clinical Documentation · Billing & Revenue Cycle · Mobile Care (In-Home PT) ·
Patient Portal · Reports · Administration · Platform / Super Admin

Audience: All staff roles, and patients using the Patient Portal

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1. Introduction & Getting Started

1.1 What is Source Motion PT

Source Motion PT is a multi-tenant Physical Therapy EMR (electronic medical record) and practice-management platform. Each physical therapy practice using the software is its own independent, isolated **organization** ("client" or "tenant") — staff and patients at one practice never see another practice's data. The platform covers scheduling, clinical documentation, billing and revenue cycle, patient/staff management, and an in-home ("Mobile Care") physical therapy workflow, plus a self-service Patient Portal.

Two kinds of people use the software:

- **Staff** at a physical therapy practice — administrators, clinical directors, therapists, PTAs, front desk/schedulers, billing specialists, and compliance officers.
- **Patients**, through a separate, simplified Patient Portal.

There is also a platform-level **Super Admin** role, used by Source Motion PT itself to operate the whole system across every practice — covered in Section 11.

1.2 Signing in for the first time (account activation)

New staff and patient accounts are set up by an administrator, who sends you a personal, one-time **activation link** (by email, or shared directly if email isn't configured for your organization).

1. Open the activation link you were given.
2. The page confirms your organization's name and the email address being activated.
3. Enter a **New password** and repeat it in **Confirm password**. Passwords must be at least **12 characters**.
4. Click **Activate account**.
5. You are signed in immediately — no separate login step is needed afterward.

NOTE

Activation links are single-use and can expire. If yours doesn't work, ask your administrator to resend it.

1.3 Logging in

1. Go to your organization's Source Motion PT web address.
2. Enter your **Login User Id** and **Login Password** (use the eye icon to reveal your typed password if you want to check it).
3. Click **Sign In**.

If your credentials are correct, you're taken into your workspace (or into a forced password-change screen first, if one is pending — see below).

1.4 Forced password changes

If an administrator resets your password for you, the next time you sign in you'll be shown a **"Set a new password"** screen instead of your normal workspace.

1. Enter and confirm a new password (12 characters minimum).
2. Click **Set new password** to continue into the app with your new password now active, or click **Log out instead** to defer — you'll be shown this same screen again next time you sign in.

There is no way to reach the rest of the application while a forced password change is pending.

1.5 Multi-factor authentication (MFA) — current status

GOOD TO KNOW

Administrators can flag an account as requiring MFA (visible as an "MFA: Required" setting when creating/editing a user), but this is currently a policy note on the account record only. The sign-in screen itself only ever asks for a Login User Id and Password — there is no authenticator-code or QR-enrollment step today. If you're told your account "requires MFA," you will not be prompted for a code at login yet.

1.6 Failed logins and account lockout

- Any failed sign-in (wrong password, or an inactive/locked/suspended/deleted account) shows the same generic message: **"Invalid username or password."** The app deliberately never reveals which of these is the real cause.
- After **5 failed attempts**, an account locks automatically for **15 minutes** (your organization may configure different numbers). The login screen gives no special indication that you're specifically locked out — it keeps showing the same generic message.
- The lock clears automatically once the 15 minutes pass, or an administrator can unlock the account immediately from the Users screen.

IF YOU'RE STUCK

If sign-in keeps failing even though you're sure the password is right, wait about 15 minutes and try again, or contact your administrator to unlock the account.

1.7 Session timeout

There is no advance warning before a session times out — you simply get returned to the sign-in screen the next time you try to do something, with a plain-language reason shown as a notice:

- **Inactivity** — about 15 minutes with no activity: *"Your session expired due to inactivity. Please sign in again."*
- **Maximum session length** — capped around 12 hours even with continuous activity.
- **Signed in elsewhere** — only one active session per account; signing in on another device ends the old one.

- Password changes/resets, and an account being locked, suspended, deactivated, or deleted, all end your session immediately with a matching message.

If you have multiple browser tabs open, they all update together the moment your session ends in any one of them.

2. Understanding Roles

Almost everything you can see and do in Source Motion PT depends on your assigned **role**. These roles are used consistently across every module described in this guide:

Role	Typical use
Organization Administrator (Admin)	Owns/runs the practice in the software: manages staff, locations, appointment types, payers, pricing, and Mobile Care settings; the only role with access to Reports and Clinic Settings.
Clinical Director	Full clinical authority (can sign any note, cosign PTAs, approve goals) plus full billing access; does not manage staff/clinic settings.
Physical Therapist (PT)	Writes and signs clinical notes, treats their own caseload, manages their own schedule.
PTA / Therapy Assistant	Writes notes and treats their own caseload, but (by default organization policy) their notes require a supervising PT/Director/Admin cosignature before they're final.
Scheduler / Front Desk	Books/moves/cancels appointments for anyone, manages patients administratively, collects payments — no clinical or full-billing access.
Billing Specialist	Full billing access (charges, claims, payments, statements, packages, superbills) — no clinical or scheduling access.
Compliance Officer	Read access to clinical charts and the audit trail for compliance review; no scheduling or billing edit access.
Patient	Self-service access to their own information only, through the separate Patient Portal.
Platform Super Admin	Source Motion PT staff who operate the whole platform across every organization; no standing access to any organization's clinical data (see Section 11).

RULE THAT APPLIES EVERYWHERE

Every screen in this guide automatically shows only what your role and your organization are entitled to see. If a button or tab described here doesn't appear for you, it's almost always because your role doesn't have that permission — not a bug.

3. Scheduling

Source Motion PT gives your clinic four connected scheduling tools: the Schedule (calendar), Appointment Types and Locations (setup), the Waitlist, and Online Booking (for patients).

3.1 The Schedule (Calendar)

What's on the screen

- View buttons: **Day**, **Week**, **Work week**, **Month**, plus **+ New event**, **← Previous**, **Today**, **Next →**.
- A **Search** box, a **Status** filter (All / Scheduled / Completed / Cancelled / No show), and a **Therapist** filter.
- A collapsible **Waitlist** panel (Section 3.4).
- The main calendar grid, and a **Selected day agenda** list below it with per-visit **Move** / **Cancel** buttons.

Book a new appointment

1. Click **+ New event**.
2. Choose the **Patient**, then a **Start** and **End** date/time.
3. Choose a **Visit type** (Initial evaluation, Follow-up, Progress, Discharge, or Telehealth).
4. Enter a **Location**, and check **Home visit** if this is an in-home visit.
5. Click **Create event**. The system automatically blocks the booking if the clinician already has a conflicting appointment.

NOTE

Therapists and PTAs can only book visits under their own name; Admins, Directors, and Schedulers can book for anyone.

Move (reschedule) an appointment

Drag and drop — click and drag a scheduled visit to a new date; the original time and duration are kept, and a conflict check runs before saving.

Move dialog — click the appointment (or its **Move** button in the day agenda), pick a new date/time, and click **Move appointment**. Only **Scheduled** visits can be moved; a visit already tied to started documentation is blocked with an explanation.

Cancel an appointment

Click **Cancel** next to the visit (or inside the Move dialog) and confirm. The visit's status changes to Cancelled and stays visible for your records — it is never deleted.

Roles

Action	Admin / Director / Scheduler	Therapist / PTA
Book/move/cancel for any patient	Yes	Own visits only
See the Schedule tab	Yes	Yes (own visits only)

Billers, Compliance officers, and patients do not see the staff Schedule tab at all.

RULES TO REMEMBER

Double-booking is prevented automatically. Only "Scheduled" visits can be moved or cancelled — completed/cancelled/no-show visits are locked. A visit tied to started documentation can't be moved. There is no recurring/repeating-visit feature — every visit is booked individually. Every booking, move, and cancellation is written to the audit log.

3.2 Appointment Types

Found under **Clinic settings** → **Appointment types** (Admin only). These are your clinic's visit-type menu, feeding both the calendar and online booking.

1. Click **+ Add type**.
2. Enter a **Name**, a **Default duration** (1–480 minutes), and an optional color tag.
3. Click **Add appointment type**.

Use **Edit** to change details, or **Deactivate/Reactivate** to retire/restore a type without losing history.

3.3 Locations

Found under **Clinic settings** → **Locations** (Admin only). Represents your clinic's physical sites (or a virtual "Telehealth" location).

1. Click **+ Add location**.
2. Enter the location name (required), phone, address, city/state/ZIP, and timezone.
3. Click **Add location**.

Use **Edit** or **Deactivate/Reactivate** as needed. A deactivated location stops appearing for new bookings and on the public booking page.

3.4 Waitlist

Staff side: expand the Waitlist panel at the top of the Schedule screen to see patients who've asked to be seen sooner, with their preferred location, date range, and notes. Book them normally on the calendar when a slot opens, then click **Mark fulfilled** (or **Remove**) — marking an entry fulfilled does not create the appointment automatically, you must still book it.

Patient side: patients join/leave the waitlist themselves from the Patient Portal (Section 8).

3.5 Online Booking

A public, no-login booking page (and an equivalent inside the Patient Portal) walks patients through: **Location** → **Service** → **Therapist** → **Date & Time** → **Your info** → **Confirm**. Full step-by-step patient instructions are in Section 8.3.

RULES TO KNOW

Patients can only cancel/reschedule online up to a clinic-configured cutoff before the visit (default 24 hours; Mobile Care home visits use their own, separately configured cutoff). Online booking must be turned on for the organization, and only the locations/services/providers explicitly opened for self-scheduling appear as options. The same double-booking protection used on the staff calendar applies to online bookings.

4. Patient Management

4.1 Finding and creating patients

Search for a patient

1. Go to the **Patients** screen.
2. Type a name or medical record number (MRN) into the search box and press Enter, or leave it blank to see your first 100 accessible patients.
3. Click **Open workspace** on the correct row.

What you can search depends on role: Therapists/PTAs see only their own assigned caseload; Scheduler and Billing roles see the whole organization; Admins, Directors, and Compliance officers see everyone.

Add a new patient

1. Click **+ New patient** (scheduling-capable roles only).
2. Enter first name, last name, and date of birth (required), plus phone/email/address/emergency contact.
3. Set the status (Active, Inactive, or Discharged) and optionally assign a therapist.
4. Clinical roles also see Diagnoses/Precautions fields here; non-clinical roles don't.
5. Click **Create patient**. The medical record number is generated automatically.

4.2 The Patient Workspace (chart)

Opening a patient shows a header (name, MRN, DOB, and — for clinical roles — diagnoses/precautions) with **Edit patient**, **Invite to portal**, and **Deactivate** actions, and a set of tabs that vary by role:

Tab	Shown to	Contains
Overview	Everyone with chart access	Compliance alerts, goal/outcome summary, clinical timeline search, and (for operations roles) the appointment list
Documentation	Clinical roles	Clinical notes, AI-assisted drafts, voice-to-note review, chart documents
Care plan	Clinical roles	Episodes of care, functional goals, outcome measures, home exercise program
Operations	Scheduling/billing roles	Scheduling, intake & consent, referrals, secure messaging, and all billing (Section 6)
Safety & audit	Admin/Director/Compliance	Read-only audit trail for this chart

4.3 Patient documents

Inside the Documentation tab's Documents card (clinical roles only):

1. Click **Choose file** and select a PDF, PNG, JPG, DOC, or DOCX (up to 15 MB).
2. Enter a Title (defaults to the filename) and optional Description.
3. Check **Share with patient portal** only if the patient should see it themselves.
4. Click **Upload document**.

Documents are private by default — sharing is an explicit, reversible toggle staff control per file.

4.4 Patient insurance

Found in Operations → Patient insurance policies (billing roles add/edit; front desk can view).

1. Click **Add insurance policy**.
2. Choose the Payer and Rank (Primary/Secondary/Tertiary).
3. Enter plan name, Member ID (required), group number, subscriber info, and effective date (required); optionally copay, coinsurance %, and deductible.
4. Click **Save insurance policy**.

Upload front/back card images per policy, run **Verify eligibility** (an on-screen reminder to check with the payer directly — there's no live clearinghouse connection), or **Terminate** a policy that's no longer active (never deleted, just marked Terminated).

4.5 Inviting a patient to the Patient Portal

1. In the patient's workspace header, click **Invite to portal** (becomes "Resend portal invite," then "Portal active" once they've signed in).
2. Confirm/edit the patient's email address.
3. Click **Send invitation**. If email isn't configured, the activation link is shown on screen for staff to relay directly.

4.6 Reviewing patient-submitted profile changes

When a patient updates their contact info through their own portal, it does **not** apply automatically — it becomes a pending request.

1. On the main Patients screen, expand the **Profile changes** panel (clinical roles only).
2. Review each pending change and click **Approve** to apply it to the chart, or **Reject** to discard it.

RULES TO REMEMBER

Nothing is ever hard-deleted — deactivating a patient just changes status. Diagnoses/precautions are clinical-only fields, never shown to front desk/billing. Patient-submitted contact-info changes always require staff approval before taking effect. Insurance-authorization visit counts do not automatically block scheduling — watch the remaining count yourself.

5. Clinical Documentation

5.1 The Documentation inbox

Lists every note across your caseload (or the whole clinic for Admins/Directors), with quick filter tabs (All, My Documentation, Unsigned Notes, Today's Notes, Drafts, Completed), a search box, and Status/Note-Type filters.

5.2 Starting a new note

1. Click **New Note** (from the inbox, a patient's chart, or a specific appointment).
2. Choose the patient if not already selected.
3. Choose the **Note Type**: Initial Evaluation, Daily Treatment Note, SOAP Note, Home Visit Note, Progress Note, Re-evaluation, Discharge Summary, or Handoff Summary.
4. Optionally link it to an existing **Episode of Care**.
5. Click **Start Documentation**.

5.3 Writing the note

The editor has a section checklist on the left (a checkmark appears once a section has content), the active section's form in the center, and reference cards on the right (previous visit, goals summary, alerts). Free-text and measurement fields **autosave** automatically a couple of seconds after you stop typing; the Interventions table and Goals form have their own explicit Save/Add buttons.

Note type	Sections
Initial Evaluation	Subjective → Objective → Assessment → Plan of Care → Goals → Signature
Re-evaluation / Progress Note	Subjective → Objective → Compare-to-Baseline/Progress → Assessment → Plan of Care → Goals → Signature
Daily Treatment / SOAP Note	Subjective → Objective/Interventions → Assessment → Plan → Signature
Home Visit Note	Subjective → Objective/Interventions → Home Visit Context → Assessment → Plan → Signature
Discharge Summary	Subjective → Objective → Assessment → Discharge Summary → Signature
Handoff Summary	Subjective → Objective → Assessment → Plan → Signature

Every note type includes a pain panel (0–10 scale, location, quality checkboxes, aggravating/relieving factors) and a clickable front/back body diagram for marking pain locations.

5.4 Setting functional goals

Inside a note (Evaluation/Re-evaluation/Progress types), the Goals tab lists Short-Term and Long-Term goals with status (Draft/Active/Met) and a form to add a new one (limitation, functional task, baseline, target, unit, target date, measurement method, wording). On the chart's **Care Plan** tab there's a fuller Goals panel that also includes an AI wording-suggestion tool. New goals always start as **Draft** and require approval (by a Therapist, Director, or Admin) before becoming Active.

5.5 Reviewing and signing a note

1. Click **Review Note** to jump to the Signature section — it lists anything still required (missing Objective, missing Plan, incomplete Plan of Care, or an overdue reassessment). These block signing until resolved.
2. Once everything required is filled in, click **Sign Note**.
3. Check the attestation box confirming you've reviewed the note and it's accurate/complete, then click **Sign Note** again to confirm.

Signing a note automatically marks its linked appointment **Completed** and counts a visit against any linked insurance authorization.

5.6 PTA notes and cosigning

ROLE RULE

By default, a PTA's signed note moves to **"Review required"** status — not yet final — and needs a supervising **Admin, Clinical Director, or PT** to click **Cosign Note** before it fully locks. A PTA can never cosign their own or another PTA's note.

5.7 Locked notes and addenda

Once fully signed, a note is completely locked — every field becomes read-only for everyone, including Admins. The only way to add or correct information afterward is an **Addendum**:

1. On a signed note, click **Create Addendum**.
2. Enter a short Reason and the addendum text.
3. Click **Save addendum**. It's appended (with your name and the date) below the original note — the original text is never altered.

5.8 AI-assist tools

Two tools exist, both explicitly labeled as drafts requiring human review:

- **AI Coding Assistance** — inside a note's Objective tab, suggests CPT codes/units from documented interventions, tagged "Suggested — Provider/Billing Review Required."
- **Goal Suggestions** — on the chart's Care Plan tab, suggests goal wording from a typed functional limitation.

Neither tool ever saves anything to the record automatically — a clinician must review and apply it.

6. Billing & Revenue Cycle

Billing work happens in three places: a patient's chart → **Operations** tab (day-to-day work for one patient), **Clinic settings** (organization-wide setup — Admin only), and **Reports** (organization-wide views — Admin only).

ROLE RULE

Full billing access (create/edit charges, build/submit claims, post payments, generate statements, sell packages, create superbills): Admin, Clinical Director, Billing Specialist. **Payment collection only** (view everything, record a front-desk payment): also Scheduler/Front Desk.

6.1 Insurance policies & authorizations

See Section 4.4 for adding a policy. **Authorizations** (a separate card) track approved visit counts — "visits used" updates automatically as notes are signed, but running low does not automatically block scheduling.

6.2 Cash-pay price list

Clinic settings → **Cash-pay prices** (Admin only) — one row per CPT/HCPCS code with a price. Charges look up this list automatically by code.

1. Enter the CPT code, a label, and the price.
2. For a Mobile Care home-visit price, tag it with a visit kind (Evaluation/Follow-up/Progress/Discharge) or check "Home visit travel fee."
3. Optionally set a deposit amount for a tagged home-visit price.
4. Click + **Add price**. Use **Deactivate/Reactivate** to retire/restore a price.

6.3 Payer directory

Clinic settings → **Payers** (Admin only).

1. Click + **Add payer**.
2. Enter the payer name (required), payer ID, electronic payer ID, phone, and address.
3. Set **Timely filing (days)** and whether authorization is required by default.
4. Click **Add payer**.

6.4 Entering a charge

1. In a patient's Operations tab → Charges → **Add charge**.
2. Choose the service date and provider (required).
3. Enter the CPT/HCPCS code (required), up to 4 modifiers, units, documented timed minutes, and the charge amount.

4. Search and add diagnosis codes.
5. If your units don't match the system's recommended count (calculated from documented minutes via the 8-minute rule), enter a units override reason.
6. Click **Save charge**.

RULES

Duplicate charges (same patient/date/provider/CPT) are blocked automatically. A charge is billed one way only — insurance *or* cash-pay, never both. Once a charge is on a submitted claim it locks; correct it by voiding and re-entering, never a silent edit.

6.5 Insurance claims

1. Operations → Claims → **Create claim from charges**. Choose the insurance policy, then add the unclaimed charges to include.
2. Click **Validate** — checks payer/policy status, diagnosis codes present, no duplicate claim, timely filing window, documentation signed, authorization coverage, and provider NPI/license status. Errors block submission; warnings don't.
3. Once the claim shows **Ready**, click **Submit claim**. (No live clearinghouse is connected — this records the claim as submitted for internal tracking; you still transmit it through your organization's outside clearinghouse/paper process.) Submitting moves every included charge from Draft to Billed.
4. Use the **"Update status to..."** dropdown + Apply to record what you learn back from the payer (Accepted, Rejected, Processing, Denied, Partial Payment, Paid, Appealed, Corrected, Closed).
5. Click **View CMS-1500 data** any time for a normalized line-item table to cross-check against your clearinghouse/paper form.

6.6 Posting payments, adjustments & transfers

1. Operations → Payments → **Post transaction**.
2. Choose the type (Insurance Payment, Patient Payment, Contractual Adjustment, Write-off, Refund, Transfer), method, amount, date, and the claim it applies to (or leave Unmatched for an ERA line you can't yet identify).
3. Click **Post transaction**.

A claim's balance is always calculated live: total charges minus payments (plus refunds) minus adjustments/write-offs — never a stored figure that can drift.

6.7 Cash-pay: superbills & front-desk payments

Superbill (for a patient to self-file with their own insurer): Operations → Billing card → **Create superbill** — enter service date, total amount, CPT/service codes, and status, then **Save superbill**. Click **View** to see the formatted document the patient would use.

Recording a front-desk payment (e.g., a copay): Operations → Billing card → **Record payment status** — amount, date, status, and the **approved payment-processor's reference number**.

IMPORTANT

Never enter a card number, expiration date, or CVV anywhere in the app — only the confirmation/reference number your organization's approved card processor gives you after running the card elsewhere.

6.8 Cash-pay packages & memberships

1. Operations → Cash-pay → **Sell package or membership**.
2. Choose Visit Package (a set number of visits) or Membership (unlimited), enter a name and price (and visit count, for a package).
3. Click **Sell package**.

Click **Record visit** each time the patient uses one (decrements the remaining count), or **Cancel** to stop it early. No insurance claim is ever involved.

6.9 Patient statements

1. Operations → Statements → enter a due date → **Generate statement**.
2. The statement combines the outstanding insurance-claim balance and the outstanding cash-pay balance into one total. Reopening an old statement always shows the numbers as of when it was generated, even if more has posted since.

6.10 AR Aging, Denial Work Queue & Billing Summary (Reports)

AR aging — filter by Payer/Claim status; shows dollar totals by age bucket and a detailed outstanding-claims table. Only submitted-or-later claims count; Draft/Ready/Validation-Error claims carry no AR exposure yet.

Denial work queue — filter by Resolution and "Overdue only"; update each denial's **Appeal status** and **Resolution** directly in the row to work it to closure.

Billing summary — a fixed 90-day window showing charges billed vs. collections (kept deliberately separate), clean claim rate, a payments breakdown, claims by status, and revenue by provider/location/payer.

All three are reachable only by Admin, Director, and Billing Specialist roles.

7. Mobile Care (In-Home Physical Therapy)

Mobile Care is Source Motion PT's in-home PT workflow: patients request a visit at home, the system helps match them to an eligible, available, nearby therapist, the visit is scheduled and carried out in the field, documented, and billed — all using the same underlying records (patients, appointments, notes, charges) as everything else in the app, not a separate system.

7.1 Turning Mobile Care on and configuring it (Organization Admin)

Reached from **Clinic settings** → **Mobile Care settings** (Admin only).

1. Check **Enable Mobile Care for this organization**. (This is on by default once your subscription includes the feature.)
2. Set a **Default visit duration** and **Maximum travel radius** (leave blank to use the platform default).
3. Set **Offer expiration time** (how long a provider has to respond to an offer), **Patient cancellation window**, and **Provider cancellation notice** (hours' notice before a provider cancellation requires a typed reason).
4. Choose which **Available services** (Initial evaluation, Follow-up, Progress, Discharge) and **Provider types** (Therapist / PTA) your organization offers via Mobile Care — leave unchecked to allow everything.
5. Choose your **Same-provider continuity preference** (prefer the patient's existing provider, or rank purely on other factors).
6. Optionally set **Matching settings** (fine-tune how heavily continuity/specialty/availability/distance/preference/caseload are weighted when ranking providers) and **Service hours** (an organization-wide window home visits must fall within).
7. Choose which **Notification settings** (events) are turned on.
8. Click **Save Mobile Care settings**.

RULE

Every setting here is optional — anything left blank falls back to a sensible platform-wide default set by Source Motion PT, and every change is logged to the audit trail.

7.2 Setting up Service Areas and Availability (per provider)

Service Areas

Clinic settings → **In-home PT areas** — defines where a provider is willing to travel.

1. Click + **Add area**, choose the provider, and enter a name (e.g., "Primary").
2. Enter a Primary ZIP code, radius in miles, city, and state.
3. Add specific ZIP codes the provider explicitly covers.
4. Save. Use **Deactivate/Reactivate** to retire/restore an area.

Availability

Each provider sets their own in-home availability (Admins/Directors can also set it for any provider in the organization).

1. Open **Mobile Care** → **Provider Availability**.
2. Click + **Add availability**.
3. Choose Availability Type (Available / Unavailable / Blocked), Day of Week (or a specific one-time date), Start/End time, and whether it's Recurring.
4. Optionally link it to a specific Service Area, set effective start/end dates, and add notes.
5. Save.

7.3 Requesting an in-home visit

Staff-entered (front desk / scheduler)

1. Open **Mobile Care** → **New Service Request**.
2. Choose the patient, requested service, preferred date/time window, visit address (with a one-time copy-in from the patient's chart address, editable), reason for visit, primary condition, specialty needed, payment method (Insurance / Self-pay / Package-Membership), and any mobility/home-access notes.
3. Submit. The request enters the matching pipeline (below).

Patient-submitted (Patient Portal)

See Section 8.4 — patients use a guided wizard in their own portal to request the same thing.

7.4 Matching, offers, and acceptance

1. From the request, staff click **Generate matches** — the system ranks every eligible provider (an ineligible provider — suspended, expired license, outside the service area, unavailable, wrong specialty — is never shown or offered) using a scored, configurable priority: continuity of care with the patient's existing provider first, then specialty match, availability, distance/service area, patient preference, and caseload.
2. Staff pick a candidate and click **Offer**. The provider sees it in their own **Visit Offers** list (a limited preview — general area and visit type, not the patient's name or exact address) and clicks **Accept** or **Decline**.
3. If declined, or the offer expires unanswered (the configured Offer Expiration Time), the system can automatically offer the next-ranked candidate.
4. Once accepted, the full visit details become visible to the provider, and the request is ready to schedule.

CONTINUITY

If the patient has an active care episode with a provider who's still eligible (active, licensed, available, within the service area), that provider is preferred automatically rather than always picking the geographically nearest one.

7.5 Scheduling the visit

Once a provider has accepted, staff (or the provider, depending on your workflow) pick an exact date and time, which creates the actual appointment — the same **Appointment** record used everywhere else in the app, just flagged as a home visit.

7.6 The provider's field-day workflow

On the day of the visit, the provider opens **Mobile Care** → **Today's Home Visits**, showing each visit's patient, time, address, and status, with buttons that change as the visit progresses:

Button	Moves status to
Start Travel	En Route
Arrived	Arrived
Start Visit	In Progress
Complete Visit	Completed

Status changes only ever move forward in this order (or to Cancelled) — the system rejects any out-of-order transition. Every status change is recorded to the audit trail.

ON LIVE LOCATION

The app does not do continuous GPS tracking of providers today. A future opt-in location-sharing feature is architected (temporary sharing only while En Route, automatically stopped on arrival, never stored long-term) but is **not turned on** — a patient does not currently see a live map or minute-by-minute location, only the visit status.

7.7 Documentation for a home visit

Once a visit is **In Progress**, the provider clicks **Start Documentation** — this opens the exact same note editor described in Section 5, using the **Home Visit Note** type (or another type, e.g.

Daily/SOAP/Progress/Discharge, as clinically appropriate). It includes the same Subjective/Objective/Assessment/Plan sections plus an extra **Home Visit Context** section: Visit Location Type, Home Safety Notes, Functional Environment, Caregiver Present, Home Exercise Education, Equipment/Assistive Device, and Environmental Barriers. Signing, locking, and addenda all work exactly as in Section 5.

7.8 Billing a home visit

Once the visit is completed and documented, staff (or, for self-pay, automatically at the org's configured price) bill it using the same Charges workflow as Section 6:

- **Service price** — looked up automatically from the organization's Cash-pay price list tagged for that visit type (e.g., "Home PT Initial Evaluation — \$175"), for self-pay; insurance visits are charged and claimed the

normal way.

- **Travel fee** — an optional additional charge, if your organization has configured one (e.g., "Optional Travel Fee — \$20").
- **Deposit / Copay / Patient responsibility** — shown to the patient (and staff) as an estimate before the visit, drawn from the organization's configured deposit amount, the patient's on-file insurance copay, or the full self-pay price as applicable — this is an estimate, not itself a separate charge; actual collection uses the same payment tools in Section 6.

No new payment system is involved anywhere in Mobile Care — it uses the same Charges, Claims, Superbills, and online-payment tools as the rest of the app.

7.9 Notifications

Patients and providers are notified automatically (in-app, and by email/SMS where enabled and where the patient hasn't opted out) at each major step: request received, therapist match found, offer created/accepted/declined, visit scheduled, a visit reminder, provider en route, provider arrived, visit completed, and visit cancelled. Messages are deliberately brief and free of unnecessary clinical detail — e.g., *"Your physical therapy home visit is scheduled for September 15 at 2:00 PM."* Patients control their own email/SMS preferences from their Portal Profile (Section 8.10), and an organization admin can turn off any specific event entirely under Mobile Care settings.

7.10 Mobile Care Home dashboard

Staff with scheduling access see a Mobile Care landing page with at-a-glance cards — Today's Home Visits, Pending Requests, Provider Offers, Active Care Episodes, Visits Needing Assignment, and License/Provider Issues — plus lists for pending requests, unassigned requests, and active care episodes. A Therapist/PTA's view is automatically narrowed to just their own visits and offers.

8. Patient Portal

The Patient Portal is a separate, simplified experience for patients, with its own sign-in and a top bar with large tabs: **Home, My Appointments, Book a Visit, In-Home PT, Forms, Documents, My Exercises, Outcome Measures, Messages, Payments, Waitlist, Profile**. It works well on both phone and desktop.

8.1 Home (dashboard)

Shows your next appointment, other upcoming visits, outstanding balance, unread message count, home exercise snapshot, any forms or outcome surveys due, recently shared documents, and notifications — each with a shortcut into the relevant full section.

8.2 My Appointments

- **Confirm a visit** — click **Confirm visit** on the appointment card.
- **Join a telehealth visit** — click **Join Visit** (active starting about 15 minutes before the scheduled time).
- **Reschedule** — click **Reschedule**, browse days, click an open time, then **Confirm new time**.
- **Cancel** — click **Cancel** and confirm.

CUTOFF WINDOW

Reschedule/Cancel buttons only appear if your visit is far enough away in time; too-close appointments show "This appointment is too close to change online — please call the clinic."

8.3 Book a Visit

1. Click **Book a Visit**.
2. Choose a **Location** (skipped if there's only one).
3. Choose a **Service**.
4. Choose a **Therapist** ("Any available" or a specific one).
5. Pick a **Date & Time** from the grouped Morning/Afternoon/Evening slots.
6. Review on the **Confirm** screen, optionally add a reason for visit, and click **Confirm booking**.

8.4 In-Home PT (Mobile Care requests)

1. Open **In-Home PT** and click to start a new request.
2. Step through: reason for visit → specialty needed → confirm your visit address (with a one-time copy-in from your chart address, editable) → preferred date → preferred time window → preferences → review & submit.
3. After submitting, you'll see a plain-language status timeline: **Request Received** → **Finding Therapist** → **Therapist Matched** → **Visit Scheduled**. Only appropriate provider information is shown (name,

credentials, specialty) — never internal matching scores.

4. Once scheduled, you'll see the date, time, provider, and general instructions.
5. You can **cancel your request** or view your upcoming home visits from the same screen, and (if pricing is configured) an estimated cost breakdown.

8.5 Forms

1. Open **Forms** and click **Start** (or **Continue/Review**) on the form you need.
2. Step through each section using **Continue/Back**; click **Save & exit** any time to finish later.
3. On the final step, click **Submit**.

Once submitted, a form's answers are locked (view-only). An "Expired" form must be redone from scratch. The digital intake form includes an Insurance step where you can enter policy details and upload photos of your insurance card.

8.6 Documents

Download anything your clinic has shared with you, or upload your own outside records (PDF, PNG, JPG, DOC, DOCX) using the file picker and an optional title/description, then **Upload**.

8.7 My Exercises (Home Exercise Program)

View your assigned exercises (with instructions, precautions, and video demonstrations where provided). Click **Mark completed** on an exercise, optionally rate pain (0–10) and difficulty (0–10), add a comment, and click **Log completion**.

8.8 Outcome Measures

Click **Start** on a pending survey, answer the questions (format varies by survey), and click **Submit**. You'll see your score immediately. Submitted surveys cannot be edited or resubmitted.

8.9 Payments

- **Pay online** — enter an amount and click **Pay now** (availability depends on your clinic having online payment connected).
- **View a statement** — click **View** in the Statements list.
- **View/print a superbill** — click **View / Download**, then **Print** to save as a PDF via your browser.

8.10 Messages

Click **New message**, choose a category (General, Appointment, Home exercise program, Billing, Clinical), enter a subject and message, and click **Send**. This is secure messaging, not email — expect a response during business hours, not instantly.

8.11 Waitlist

Optionally choose a location, enter your earliest available date, add notes, and click **Join waitlist**. Click **Remove** on your entry to leave it. Joining does not book anything automatically — the clinic calls you if a matching opening appears.

8.12 Profile

Contact information (phone, email, address, emergency contact) — edits are submitted for staff review and only take effect once approved; the fields lock while a request is pending.

Pharmacy & communication preferences — save instantly, no approval needed: pharmacy info, preferred contact method, whether to be emailed for new secure messages, and whether to receive text updates about in-home PT visits.

9. Reports

ROLE RULE

Reports is visible only to the **Organization Administrator** role — every other role, including the platform Super Admin, is redirected away and never sees a Reports entry in their navigation.

Four tabs: **Operational report**, **Billing summary**, **AR aging**, **Denial work queue**.

- **Operational report** (fixed 30-day window, no filters) — new patients, notes signed/unsigned, outcomes recorded, appointments by status, overdue reassessments, and active caseload by provider.
- **Billing summary** (fixed 90-day window, no filters) — see Section 6.10.
- **AR aging** (filterable by payer/claim status) — see Section 6.10.
- **Denial work queue** (filterable, and directly editable per row) — see Section 6.10.

NOTE

There is currently no export/download/print button on the Reports page — it's for on-screen viewing only.

10. Administration (Organization Admin)

Reached from the Home screen's **Administration** tile (Admin role only).

10.1 Managing your organization's staff

Create a new staff user

1. Open **Administration** → **Users** → **Create user**.
2. Enter User ID, email, first/last name.
3. Choose an **Access Level** (role).
4. For Physical Therapist or PTA/Assistant, also enter License Number, Issuing State, and License Expiration Date (required for licensed roles).
5. Set a temporary password, leave **Require MFA** checked unless you have a reason not to, and click **Create user**.

Edit, deactivate, suspend, unlock, delete, or restore a user

- **Edit user** (⋮ menu) — update name/email/username/credential/role/MFA. You cannot edit your own role, MFA, or reset your own password from here.
- **Deactivate** — blocks sign-in, reversible any time, no reason required.
- **Suspend** — blocks sign-in immediately, requires a typed reason, does not expire on its own (an admin must explicitly reactivate).
- **Unlock Account** — clears a failed-login lockout immediately.
- **Delete User** — blocks sign-in and hides the account, but preserves all history; reversible via **Restore User** from the "Deleted" filter.
- **Sign Out User From All Devices** — revokes every active session for that user immediately.

Reset a password

Open **Edit user**, type a new temporary password in the "New Password" field, and save — the user is required to set their own password on next login.

10.2 Managing clinician licenses

From **Edit user** on a Therapist/PTA, the "Professional Licenses" section lets you add a license (number, state, type, issue/expiration dates), upload a document, and — once uploaded — click **Verify license** with optional notes. A fully expired license automatically suspends that provider's clinical access.

10.3 Locations, Appointment Types, Payers

See Sections 3.2, 3.3, and 6.3.

10.4 Mobile Care settings

See Section 7.1.

RULE

Everything an Organization Administrator does is automatically confined to their own organization — they cannot see or affect any other practice's data or users, even by URL manipulation; this is enforced by the server, not just hidden in the interface.

11. Platform Administration (Super Admin)

Platform Super Admins (Source Motion PT staff) land on a platform-wide Home dashboard (organization/user/patient counts, credential alerts, recent activity, a failed-login trend chart) and an **Administration** hub with four buttons: **Clients**, **Users**, **Credentials**, **Mobile Care Defaults**.

11.1 Client (tenant organization) management

Create a new client

1. **Clients** → **Add client**.
2. Enter facility information (name, email, address), account settings (subscription tier, timezone), and the primary administrator's name/email.
3. Click **Create client** — an invitation is emailed (and shown on screen as a shareable link, in case email delivery isn't configured).

Manage a client's subscription

Open the client → **Subscription** tab → choose a Plan, Status, Billing cycle, and Provider seats, adjust individual feature checkboxes if this client needs a custom mix, then **Save subscription**.

Suspend / reactivate / archive a client

- **Suspend** — requires a written reason; blocks sign-in for every user/patient at that org immediately; fully reversible.
- **Reactivate** — restores access immediately.
- **Archive** — requires a written reason; treated as permanent from the admin interface (Source Motion PT support would need to restore it).

11.2 Requesting time-boxed privileged access

KEY RULE

Super Admins have **no standing access** to any organization's patient charts or Mobile Care operational data. Every view requires an explicit, reasoned, time-boxed request first.

1. Open the client → **Clinical Access** tab → **Request access**.
2. Enter a Reason (required) and choose a Duration (1, 4, or 24 hours).
3. Submit. While active, you can view that client's Patients list (and a Mobile Care overview, if enabled) — every chart view and every action is individually audited against this specific grant.
4. Click **Revoke now** to end your own access early, or let it expire automatically.

The tab's "Access history" shows every grant ever requested for that client — who, when, why, and its outcome.

11.3 Platform-wide Users, Credentials, and Mobile Care Defaults

Users — identical to Section 10.1, but spans every client (with a Client column and filter) instead of just one organization.

Credentials — a platform-wide, read-only license-expiration dashboard across every organization (Needs Attention / Expired / Critical / Expiring Soon / All Licenses), with a column showing whether an expired license has already auto-suspended that provider's account.

Mobile Care Defaults — sets the platform-wide starting values (visit duration, travel radius, offer expiration, cancellation windows, continuity preference) that any organization's own Mobile Care settings (Section 7.1) can override; a change here only affects organizations that haven't set their own value for that particular setting.

12. Appendix: Security & Data Handling Notes

- **Tenant isolation** — every organization's data is completely separated from every other organization's; this is enforced by the server on every request, not just hidden in the interface.
- **Audit trail** — nearly every meaningful action (logins, record changes, chart access, billing actions, privileged-access grants) is recorded with who did it and when. Audit records deliberately never include clinical narrative text.
- **Nothing is silently deleted** — patients, users, insurance policies, service prices, and clients are deactivated/suspended/archived rather than removed, and are generally reversible (except an archived client, which requires Source Motion PT support to restore).
- **Signed clinical notes are permanently locked** — corrections always happen via a clearly attributed addendum, never a silent edit.
- **No cardholder data is ever stored** — only a payment processor's own confirmation/reference number is entered anywhere in the app.
- **No live GPS tracking of providers exists today** — the architecture for a future, opt-in, travel-only tracking feature has been designed but is not active.
- **No live insurance-clearinghouse connection exists today** — claim submission, status checks, and eligibility checks are internal tracking; staff still transmit claims and verify eligibility through their own outside processes.